

Vaginal Discharge - Female (Child)

Algorithm 1370 | Child (12-17 years) | Gender at birth: Female

Definition

- UAT-only assessment of vaginal discharge in female patients aged 12 years and older; not approved for real-patient or production use.
- Child patient.
- Female.

Initial assessment

1. [OPEN_TEXT] How old is the patient, and can they speak privately and safely? For anyone under 18, use the Qatar-approved safeguarding pathway for possible coercion, abuse, exploitation, or an unsafe accompanying person.
2. [OPEN_TEXT] Describe the color, smell, amount, duration, itching or soreness, and whether a retained tampon or other vaginal foreign body is possible.
3. [YES_NO] Is pregnancy possible or confirmed, or is there fever, pelvic or lower-abdominal pain, vomiting, pain urinating, bleeding between periods or after sex, recent delivery/procedure, or immunocompromise?
4. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Sores

Are genital sores or open lesions the main concern rather than vaginal discharge?

Open or non-healing lesions belong in the Sores pathway.

1. Disposition level 70

Has the discharge changed color, smell, texture, or amount; is there itching, soreness, urinary pain, bleeding, possible pregnancy, pelvic pain, fever, vomiting, a possible retained foreign body, recent delivery/procedure, immunocompromise, or concern for sexual abuse or exploitation?

NHS.UK vaginal discharge guidance recommends prompt clinical review for these changes, which can indicate infection.

1. Disposition level 15

For a non-pregnant adult already familiar with their usual discharge, is this unchanged, typical discharge with none of the urgent, safeguarding, or vulnerability features above?

NHS.UK guidance describes normal vaginal discharge as expected and usually nothing to worry about.

Advice

Urgent vaginal discharge review

- Arrange same-day in-person assessment through the Qatar service approved for this UAT environment. Do not diagnose an STI or recommend an antimicrobial, antifungal, vaginal preparation, or removal of a possible foreign body by telephone. If severe or rapidly worsening pelvic pain, collapse, heavy bleeding, severe illness, or pregnancy with an immediate threat develops, call Qatar 999 and do not self-drive.
- Call Back If: pelvic pain, fever, vomiting, heavy bleeding, faintness, or severe illness develops; pregnancy is possible or confirmed; safeguarding concern or inability to speak privately.

Home care for normal discharge

- Wash the area gently with warm water and mild, non-perfumed soap. Avoid perfumed soaps, deodorants, scented wipes, and douching.
- Call Back If: color, smell, or texture changes; itching, soreness, or pain develops.

First aid

References

- NHS.UK, "Vaginal discharge", <https://www.nhs.uk/conditions/vaginal-discharge/> (page last reviewed 15 February 2024)

Provenance

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